Metabolic Assessment Form™ – Blood Chemistry Key

CATEGORY I: Colon

Eosinophils **increased**

Check immune health
Consider ordering a GI stool test or Cyrex™ Laboratories Array 2 for intestinal permeability

CATEGORY II: Intestinal Barrier

Protein and globulin **increased or decreased**

CATEGORY III: Chemical Tolerance

Protein and globulin **increased or decreased** associated with intestinal permeability
Depressed 25-hydroxyvitamin D levels associated with reduced regulatory T-cell support

CATEGORY IV: Stomach

(Check for Hypochlorhydria)

Protein and globulin **increased or decreased**
BUN **increased or decreased**
Phosphorus **decreased**

Check for possible H. pylori infection, which may alter acid production:

WBC **increased or decreased**
Neutrophils **increased**

CATEGORY V: Stomach

(Check for Hyperacidity)

Protein and globulin **increased or decreased**
BUN **increased or decreased**
Phosphorus **decreased**

Check for possible H. pylori infection, which may alter acid production:

WBC **increased or decreased**
Neutrophils **increased**

CATEGORY VI: Pancreas

Pancreatic enzyme deficiency shows up many times as insulin resistance

Check for pancreatic enzyme deficiency
Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars

Usually only symptoms associated and not associated with blood chemistry markers

CATEGORY VII: Small Intestine

Usually only symptoms associated and not associated with blood chemistry markers

CATEGORY VIII: Biliary

GGTP, alkaline phosphatase—don't usually show up until obstruction

MAF symptoms may be due to thick bile, pre-gallstones, or gallstones

Some individuals may have no symptoms because they avoid foods that cause them

CATEGORY IX: Hepatic Detoxication

Usually only symptoms associated and not associated with blood chemistry markers

May be suggested from functional elevation of SGOT, SGPT, and/or GGTP

CATEGORY X: Sugar Metabolism

(Check for Hypoglycemia)

Glucose **decreased**
LDH **decreased**

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The lab tests and blood markers referenced on this form are solely intended for nutritional evaluation within the reference range. Blood test markers that are out of range may require pharmaceutical intervention or other medical treatment and will require that the individual be seen by a medical practitioner with a scope of practice to diagnose, treat, and cure disease.

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CATEGORY XI: Peripheral Utilization of Sugars

(Check for Insulin Resistance)

Glucose **increased**
Triglycerides **increased**
Total cholesterol and LDL **increased**
HDL **decreased**
Uric acid **increased**

CATEGORY XII: Adrenal

(Check for Hypofunction)

Sodium and potassium **changes**

Check Category X on Metabolic Assessment Form™ results to assess sugar metabolism

CATEGORY XIII: Adrenal

(Check for Hyperfunction)

Sodium and potassium **changes**

Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars

CATEGORY XIV: Electrolyte and pH Balance

Sodium, potassium, chloride, calcium, and magnesium levels
CO_2 levels
Anion gap

CATEGORY XV: Thyroid

(also check for Hypofunction)

TSH **increased**
T3 Uptake
Total T4
Total T3
FTI
Thyroid antibodies **increased**

CATEGORY XVI: Thyroid

(Check for Hyperfunction)

TSH **decreased**
T3 Uptake
Total T4
Total T3
FTI
Thyroid antibodies **increased**

CATEGORY XVII: Prostate (Males Only)

PSA
Free PSA

CATEGORY XVIII: Andropause (Males Only)

Check for insulin resistance
Check for systemic inflammation
Check for GI infections
Check for neurodegeneration
Check hormone levels (testosterone:estrogen ratio)

CATEGORY XIX: Menstruating Women

Check for systemic inflammation
Check hormone levels

CATEGORY XX: Menopausal Women

Check for systemic inflammation
Check hormone levels

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(Check for Insulin Resistance)

Glucose **increased**
Triglycerides **increased**
Total cholesterol and LDL **increased**
HDL **decreased**
Uric acid **increased**

CATEGORY XII: Adrenal

(Check for Hypofunction)

Sodium and potassium **changes**

Check Category X on Metabolic Assessment Form™ results to assess sugar metabolism

CATEGORY XIII: Adrenal

(Check for Hyperfunction)

Sodium and potassium **changes**

Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars

CATEGORY XIV: Electrolyte and pH Balance

Sodium, potassium, chloride, calcium, and magnesium levels
CO_2 levels
Anion gap

CATEGORY XV: Thyroid

(also check for Hypofunction)

TSH **increased**
T3 Uptake
Total T4
Total T3
FTI
Thyroid antibodies **increased**

CATEGORY XVI: Thyroid

(Check for Hyperfunction)

TSH **decreased**
T3 Uptake
Total T4
Total T3
FTI
Thyroid antibodies **increased**

CATEGORY XVII: Prostate (Males Only)

PSA
Free PSA

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Check for neurodegeneration
Check hormone levels (testosterone:estrogen ratio)

CATEGORY XIX: Menstruating Women

Check for systemic inflammation
Check hormone levels

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Check for systemic inflammation
Check hormone levels

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Metabolic Assessment Form™ Interpretation

The Metabolic Assessment Form™ (MAF) is a tool you can use as part of your intake and assessment process. It is organized to help you quickly and easily identify issues and their subjective importance to your patients. The MAF is used to develop dietary, nutritional, and lifestyle strategies, helps determine which blood tests should be ordered, correlates the patient's symptoms to blood chemistry laboratory patterns, and helps monitor the patient's outcomes. It is not designed to offer a diagnosis for any condition. Further determinations will be necessary to evaluate the value of the findings from this form.

The MAF Keys

The MAF has two keys, the MAF Blood Chemistry Key and the MAF Nutritional Key. The blood chemistry key notes blood test markers related to the category and may offer suggestions for additional testing.

The nutritional key suggests protocols for each category. The nutritional protocols are used to support various body systems. **The products listed are only intended as support for natural metabolic processes that are under temporary stress or to address an additional demand for nutrients. These recommendations have not been evaluated by the Food and Drug Administration and should not be construed as claims to treat, cure, or prevent disease, as they are not intended to act as drugs or to replace any drug prescribed by a physician.** We believe that such an act may be detrimental to the health and well-being of an individual and that dietary supplements should only be used in a supportive role in such conditions.

The MAF Parts

The assessment form has four parts that the patient completes, which includes:

- Part I: Health Concerns – The patient lists five major health concerns
- Part II: Categories – Gather subjective data to prioritize and recognize system concerns for different metabolic categories
- Part III: Foods – Recognize dietary habits that may alter metabolism
- Part IV: Medications – Recognize medications that may alter metabolism

Part I: Health Concerns

Part I is the most important category of the questionnaire form, as it discusses an individual's five major health concerns in order of importance. This helps the clinician to understand what the individual expects from the clinician. The clinician can also use this section to help the individual understand what systems are associated with his or her symptoms.

Part II: Categories

The questions in each category relate to a body part, function, or physiological mechanism. The symptoms are subjective. The categories are:

- Colon
- Intestinal Barrier
- Chemical Tolerance
- Stomach (Two Categories)
- Pancreas
- Small Intestine
- Biliary
- Hepatic Detoxication
- Sugar Metabolism
- Peripheral Utilization of Sugars
- Adrenal (Two Categories)
- Electrolyte and pH Balance
- Thyroid (Two Categories)
- Prostate
- Andropause (Males Only)
- Menstruating Women
- Menopausal Women

Category I – Colon Interpretation

This category screens for symptoms associated with gastrointestinal dysfunction. The gastrointestinal symptoms may be due to hypochlorhydria, pancreatic insufficiency, biliary stasis, dysbiosis, yeast overgrowth, gastric or duodenal ulcer, etc. If the individual has symptoms associated with this category, a comprehensive evaluation of the gastrointestinal tract should be conducted.

Unless the gastrointestinal tract imbalance is addressed, all therapies to support imbalances at any other level will likely be unsuccessful.

- Feeling that bowels do not empty completely
- Lower abdominal pain relieved by passing stool or gas
- Alternating constipation and diarrhea
- Diarrhea
- Constipation
- Hard, dry, or small stool
- Coated tongue or "fuzzy" debris on tongue
- Pass large amount of foul-smelling gas
- More than 3 bowel movements daily
- Use laxatives frequently

Further Assessment of Category I – Colon

Further assessment is required in the following areas:

- Diet and Lifestyle
  - Check for inflammatory foods
  - Check for lack of water
  - Check for sedentary lifestyle
- Motility Issue
  - Check for brain-gut axis imbalance
  - Check for low thyroid function
- Pathogenic
  - Check for parasites—elevated eosinophils
- Autoimmune
  - Check for ASCA and/or ANCA antibodies
- Celiac
  - Check for transglutaminase antibodies
- Intestinal Permeability
  - Cyrex® Laboratories Array 2 Intestinal Antigenic Permeability Screen—occludin/zonulin antibodies, actomyosin antibodies, lipopolysaccharide antibodies

MAF Blood Chemistry Key Association Category I – Colon

- Check eosinophiles (may increase with parasitic infection)
- Check immune health
- Consider ordering a GI stool test or Cyrex® Laboratories Array 2 for intestinal permeability

MAF Nutritional Key Category I – Colon*

*Nutritional Support for the Colon:**

4-R Program: Remove, Replace, Reinoculate, and Revive

Supplements for the "Remove" phase:*

1. GI-Synergy™◊ (K64), (K83): 1 packet once a day.

Supplements for the "Replace" phase:*

1. BileMin™◊ (K11): 1-2 capsules with meals.
2. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

Supplement for the "Reinoculate" phase:*

1. Strengtia™◊ (K61): 1-3 capsules a day.

   Alternative Formula
   - Sibiotica™◊ (K97): 1-3 capsules twice a day.

Supplements for the "Revive" phase:*

1. RepairVite™◊ (K60), (K63): 1 scoop, with up to 8 ounces of water, 1-2 times a day.

   Alternative Formulas
   - RepairVite-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, twice a day.
   - RepairVite-GT™◊ Plus (K120): 1 scoop, with up to 8 ounces of water, once a day.
2. Strengtia™◊ (K61): 1-3 capsules a day.

   Alternative Formula
   - Sibiotica™◊ (K97): 1-3 capsules twice a day.

Please consider the Mucosal Health & Food Tolerance Program™ (from the corresponding guide)

Note: Consider pharmaceutical management to eradicate infections. The 4-R Program—which stands for remove, replace, reinoculate, and revive—should be considered to support gastrointestinal health.*

"Remove" includes supporting the microbial environment.* Individuals should work with their primary care physician to address the need to remove pathogenic organisms; however, one can use nutrients and herbs to address the terrain and support an environment in which it is less favorable for the development of bacteria, etc.* GI-Synergy™◊ (K64) combines three formulas that are each designed to support the intestinal terrain.* H-PLR™◊ (K32),

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

Parastonil™◊ (K59), and Yeastonil™◊ (K58) are combined into one easy-to-use package for the individual.

"Replace" includes supporting the digestive system with enzymes and hydrochloric acid (EnzymixPro™◊ [K99]) and biliary support (BileMin™◊ [K11]).* EnzymixPro™◊ (K99) is a source of hydrochloric acid and enzymes that support digestive processes.* BileMin™◊ (K11) is a source of beet concentrate, taurine, vitamin C, dandelion root, milk thistle extract, ginger, and lecithin used to support the biliary system.*

To "Reinoculate" involves replenishing the gastrointestinal tract with friendly bacteria, such as *Lactobacillus and Saccharomyces boulardii.* Strengtia™◊ (K61) is a formula that offers targeted probiotics to help support the intestines.* It includes prebiotic support for enhanced potency of the probiotics. The ingredients support the bacterial environment and the intestinal immune system.*

"Revive" includes nutritional support for the gastrointestinal lining.* When reviving the gastrointestinal tract, nutrients such as L-glutamine, gamma oryzanol, aloe leaf extract, flavonoids, and deglycyrrhizinated licorice are known to support the intestinal wall structure and function.* RepairVite™◊ (K60), (K63) supports the maintenance of the intestinal tract and intestinal lining.*

At times, an individual may need a fiber supplement as a bulking agent to support bowel movements.* Fibromin™◊ (K25) offers a variety of soluble and insoluble fibers, including guar gum, pectins, lignans, cellulose, and enzymes.

Category II – Intestinal Barrier Interpretation

This category screens for symptoms associated with intestinal permeability, also known as "leaky gut." The functional laboratory markers for intestinal permeability of abnormal (high or low) functional changes of total protein and globulin are identical to hypochlorhydria, so symptom evaluation may help differentiate the appropriate mechanisms. Intestinal permeability can be appropriately identified by the Cyrex® Laboratories Array 2. The profile will measure antibodies for lipopolysaccharides, actomyosin, and occludin/zonulin. If any of those markers are positive, intestinal permeability is objectively confirmed.

If intestinal permeability is confirmed, then the mechanism for its development must be identified.

Common causes for intestinal permeability are undiagnosed gluten intolerance or celiac disease. Intestinal permeability can be evaluated by Cyrex® Laboratories Array 3 serum laboratory test. The use of medications and antibiotics, as well as alcohol abuse, systemic inflammation, and elevated stress responses may also be causative factors in intestinal permeability.

- Increasing frequency of food reactions
- Unpredictable food reactions
- Aches, pains, and swelling throughout the body
- Unpredictable abdominal swelling
- Frequent bloating and distention after eating

Further Assessment of Category II – Intestinal Barrier

Further assessment is required in the following areas:

- Check for intestinal permeability with Cyrex® Laboratories Array 2 for evaluation of:
  - Occludin/zonulin antibodies
  - Actomyosin antibodies
  - Lipopolysaccharide antibodies
- Identify mechanism of intestinal permeability

MAF Blood Chemistry Key Association Category II – Intestinal Barrier

Protein and globulin increased or decreased.

MAF Nutritional Key Category II – Intestinal Barrier*

*Nutritional Support for the Intestinal Barrier:**

1. RepairVite™◊ (K60), (K63): 1 scoop, with up to 8 ounces of water, 1-2 times a day.

   Alternative Formulas
   - RepairVite-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, once a day.
   - RepairVite-GT™◊ Plus (K120): 1 scoop, with up to 8 ounces of water, once a day.
2. EnteroVite™◊ (K100): 1 capsule 3 times a day.
3. Trizomal™◊ Glutathione (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.

Additional Support Considerations:

1. GI-Synergy™◊ (K64), (K83): 1 packet, twice a day, with a meal.
2. Strengtia™◊ (K61): 1-3 capsules a day.

   Alternative Formula
   - Sibiotica™◊ (K97): 1-3 capsules twice a day.

MAF Nutritional Key
Category II – Intestinal Barrier* (Cont.)

3. Turmero™◊ Active (K75), (K109): 5 mL (approx. 1 teaspoon) 1-2 times a day.
4. Resvero™◊ Active (K76), (K110): 5 mL (approx. 1 teaspoon) once a day.
5. AD-Pro™◊ (K119): 1 softgel, once a day, with a meal.

   Alternative Formulas
   - ADEK-Pro™◊ (K102): 1-3 softgels once a day.
   - Ultra-D 5000™◊ (K78): 1-4 teaspoons, once a day, with meals.
   - Ultra-D Complex™◊ (K35): 1-4 teaspoons, once a day, with meals.

For additional vitamin D, consider Liqua-D™◊ (K87): 1 drop once a day.

Please consider the RepairVite™◊ Program (from the corresponding guide).

Nutritional support for the gastrointestinal system includes the RepairVite™◊ (K60), (K63) powder and the associated dietary recommendations provided by the RepairVite™ Program Guide.* The use of GI-Synergy™◊ (K64) is also recommended.* Strengtia™◊ (K61), a multi-use probiotic with arabinogalactan, is also useful for nutritional support of intestinal flora.* Ultra-D Complex™◊ (K35) or Ultra-D 5000™◊ (K78) can be used as a nutritional support for the intestinal barrier and for the immune system.*

Category III – Chemical Tolerance Interpretation

This category screens for symptoms associated with a loss of chemical tolerance. A loss of chemical tolerance occurs with a breakdown of the intestinal barrier, the blood-brain barrier, or the pulmonary barrier, as well as from the disruption of regulatory T-cells. It may also occur from environmentally induced depletion of antioxidant systems. There are no laboratory patterns associated directly with a loss of chemical tolerance; however, patterns for intestinal permeability and vitamin D status should be evaluated, as discussed earlier.

- Intolerance to smells
- Intolerance to jewelry
- Intolerance to shampoo, lotion, detergents, etc.
- Multiple smell and chemical sensitivities
- Constant skin outbreaks

Further Assessment of
Category III – Chemical Tolerance

Further assessment is required in the following areas:

- Check for loss of chemical tolerance and self-tolerance
  - Check for failure of regulatory T-cells (vitamin D and glutathione)
  - Check for barrier permeability
  - Check for failure of glutathione recycling system
- Loss of chemical tolerance is usually associated with loss of self-tolerance and autoimmunity
- Cyrex® Laboratories Array 11 Chemical Immune Reactivity Screen

MAF Blood Chemistry Key Association
Category III – Chemical Tolerance

- Protein and globulin increased or decreased associated with intestinal permeability
- Depressed 25-hydroxyvitamin D levels associated with reduced regulatory T-cell support

MAF Nutritional Key
Category III – Chemical Tolerance*

*Dietary and Nutritional Support for the Immune System:**

1. Repair & Clear™ Program (basic or comprehensive): As directed by the Repair & Clear™ Program Guide.

Nutritional support for chemical tolerance includes strategies to support the glutathione recycling system, barrier system, and hepatic biotransformation function.* A comprehensive nutritional and dietary program is listed in the Repair & Clear™ Program Guide.

Category IV – Stomach Interpretation

(Also check for Hypochlorhydria)

Hypochlorhydria is a condition in which there is an inadequate production of hydrochloric acid (stomach acid). Although some free acid is present in gastric juice, it is not adequate to digest protein efficiently. When hypochlorhydria exists, there is inadequate acid to sterilize stomach content, which increases the susceptibility of infections by pathogenic bacteria, yeast, and parasites. The typical symptoms of hypochlorhydria include gas and bloating after meals. Gas and bloating take place because foods are not digested properly, and therefore, undergo fermentation. Offensive

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

breath is common due to protein putrefaction. The sense of fullness in the stomach after meals that takes place is due to excess gas from fermentation. Hypochlorhydria is a condition that should be treated with the utmost priority because it is the first step in a chain of digestive reactions that leads to the release of bile and pancreatic enzymes. Hypochlorhydria also leads to nutritional malabsorption of essential nutrients—such as zinc, vitamin B_{12}, manganese, and calcium—which require an acidic environment for absorption. Hypochlorhydria may be caused by deficiencies of zinc and B vitamins, hypothyroidism, food sensitivities, prolonged exposure to pathogenic organisms, *H. pylori* inoculation, or a loss of parietal cell function from aging.

It is important to note that chronic hypochlorhydria is often secondary to a subclinical *H. pylori* infection. In these cases, there are no gastric ulcer symptoms, just patterns that represent decreased HCl production. The *H. pylori* organisms furrow into the gastric lining and shut down parietal cell release of HCl. Therefore, with any case of chronic hypochlorhydria, always test for *H. pylori*.

- Excessive belching, burping, or bloating
- Gas immediately following a meal
- Offensive breath
- Difficult bowel movements
- Sense of fullness during and after meals
- Difficulty digesting proteins and meats; undigested food found in stools

Further Assessment of Category IV – Stomach

Further assessment is required in the following areas:

- Check for parietal cell degeneration from aging
- Check for symptoms of failure of brain-gut axis
- Check for low thyroid function
- Check for intestinal inflammation:
  - Check for infections – *H. pylori*
  - Check for gluten/celiac disease
  - Check for intestinal permeability

MAF Blood Chemistry Key Association Category IV – Stomach

- Check for possible hypochlorhydria:
  - Protein and globulin increased or decreased
  - BUN increased or decreased
  - Phosphorus decreased
- Check for possible *H. pylori* infection, which may alter acid production:
  - WBC increased or decreased
  - Neutrophils increased

MAF Nutritional Key Category IV – Stomach

Nutritional Support for Healthy Stomach Acid Levels and the Gastrointestinal System:

1. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

   Alternative Formulas
   - HCL Pro-Zyme™◊ (Z26): 1-2 tablets with each meal.
   - SuperDigest-Zyme™◊ (Z22): 1-2 tablets with each meal.
2. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

*Nutritional Support for pH Balance and the Gastrointestinal System:**

1. H-PLR™◊ (K32): 1-2 capsules with each meal.
2. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

   Alternative Formulas
   - HCL Pro-Zyme™◊ (Z26): 1-2 tablets with each meal.
   - SuperDigest-Zyme™◊ (Z22): 1-2 tablets with each meal.
3. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

To support levels of hydrochloric acid, EnzymixPro™◊ (K99) should be considered.* EnzymixPro™◊ (K99) supplies a high level of betaine HCl and includes enzymes to help support the digestion of certain sugars, starches, fibers, proteins, and fats.*

Category V – Stomach Interpretation

(Also check for Hyperacidity)

Hyperacidity is often associated with hypochlorhydria that has progressed to the point in which the lining of the stomach has been destroyed. In cases of hypochlorhydria, proteins are not digested due to a lack of HCl and subsequently putrefy. The putrefaction of protein creates a very acidic environment in the stomach that may lead to stomach lining damage and ulcers. If the individual has both categories of hypochlorhydria and hyperacidity, always suspect that the cause of hyperacidity is second to hypochlorhydria and protein putrefaction.

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Apex Energetics™ products are intended for nutritional use and health maintenance only and should not replace or delay the use of any medication.

- Stomach pain, burning, or aching 1-4 hours after eating
- Use of antacids
- Feel hungry an hour or two after eating
- Heartburn when lying down or bending forward
- Temporary relief by using antacids, food, milk, or carbonated beverages
- Digestive problems subside with rest and relaxation
- Heartburn due to spicy foods, chocolate, citrus, peppers, alcohol, and caffeine

Further Assessment of Category V – Stomach

Further assessment is required in the following areas:

- Check for *H. pylori*
- Check for high aspirin use
- Check for stress-induced permeability
- Check for gastric cancer
- Check for autoimmunity:
  - Parietal cell antibodies
  - Intrinsic factor antibodies

MAF Blood Chemistry Key Association Category V – Stomach

- Check for hyperacidity:
  - Protein and globulin increased or decreased
  - BUN increased or decreased
  - Phosphorus decreased
- Check for possible *H. pylori* infection, which may alter acid production:
  - WBC increased or decreased
  - Neutrophils increased or decreased

MAF Nutritional Key Category V – Stomach*

*Nutritional Support for pH Balance, Healthy Stomach Acid Levels, and the Gastrointestinal System:**

1. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

   Alternative Formulas
   - HCL Pro-Zyme™◊ (Z26): 1-2 tablets with each meal.
   - SuperDigest-Zyme™◊ (Z22): 1-2 tablets with each meal.
2. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

*Nutritional Support for Gastrointestinal Health:**

1. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.
2. H-PLR™◊ (K32): 1-2 capsules with each meal.

*Nutritional Support for pH Balance and the Gastrointestinal System:**

1. H-PLR™◊ (K32): 1-2 capsules with each meal.
2. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

   Alternative Formulas
   - HCL Pro-Zyme (Z26): 1-2 tablets with each meal.
   - SuperDigest-Zyme (Z22): 1-2 tablets with each meal.
3. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.
4. Myco-Zyme™◊ (Z13): 1-2 capsules with meals.

*Nutritional Support for pH Balance and the Adrenals:**

1. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

To support levels of hydrochloric acid and digestion, consider using EnzymixPro™◊ (K99).* Many individuals will notice supported digestion immediately when taking the enzyme they need to break down protein and prevent protein putrefaction.* Some of these individuals may, however, have increased issues with the ingestion of HCl. If an individual does not have a good response with EnzymixPro™◊ (K99), consider using Gastro-ULC™◊ (K29), which is a source of deglycyrrhizinated licorice, L-glutamine, flavonoids, bismuth, gamma oryzanol, chlorella, rhubarb, and mastic gum.* After 30 days, individuals can consider taking a small serving of HCl to support the stomach.* To support the bacterial environment in the stomach, H-PLR™◊ (K32) should be considered.* Cortisol from stress thins the lining of the stomach and makes an individual more susceptible to changes in the acidic environment.

Category VI – Pancreas Interpretation

Pancreatic insufficiency will cause symptoms similar to hypochlorhydria, but the symptoms are generally experienced after starch ingestion versus protein ingestion. Most people who have pancreatic insufficiency have it

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

secondary to hypochlorhydria. If the bolus does not become acidic enough once it leaves the stomach and enters the duodenum, it will not trigger the release of pancreatic enzymes and will hence induce secondary pancreatic insufficiency. Pancreatic insufficiency also commonly occurs from chronic gastrointestinal inoculation with a pathogen. In these cases, the pancreas is called into action to release enzymes to support the GI tract due to its anti-inflammatory and proteolytic nature. Primary pancreatic insufficiency is often due to diets high in starch and with individuals who have some type of blood sugar regulation disorder.

- Difficulty digesting roughage and fiber
- Indigestion and fullness last 2-4 hours after eating
- Pain, tenderness, soreness on left side under rib cage
- Excessive passage of gas
- Nausea and/or vomiting
- Stool undigested, foul smelling, mucus like, greasy, or poorly formed
- Frequent loss of appetite

Further Assessment of Category VI – Pancreas

Further assessment is required in the following areas:

- Check for excess-starch diet
- Check for failure of brain-gut axis
- Check for low thyroid
- Check for intestinal inflammation:
  - Check for infections
  - Check for gluten sensitivity/celiac disease
  - Check for intestinal permeability

MAF Blood Chemistry Key Association Category VI – Pancreas

- Pancreatic enzyme deficiency shows up many times as insulin resistance
- Check for pancreatic enzyme deficiency
- Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars
- Usually only symptoms associated and not associated with blood chemistry markers

MAF Nutritional Key Category VI – Pancreas*

*Nutritional Support for the Pancreas:**

1. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.
2. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

*Nutritional Support for the Pancreas and Healthy Stomach Acid Levels:**

1. EnzymixPro™◊ (K99), (K99A): 1-2 capsules with each meal.

   Alternative Formula
   - HCL Pro-Zyme™◊ (Z26): 1-2 tablets with meals.

   Additional Formula
   - SuperDigest-Zyme™◊ (Z22): 1-2 tablets with meals.

*Dietary and Nutritional Support for the Small Intestine:**

1. ClearVite™◊ Basic Program: Three-week intestinal support program as directed by the ClearVite™◊ Basic Program Guide.
2. Gastro-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

Category VII – Small Intestine Interpretation

The body intentionally keeps the bacterial amount low in the small intestine. There are several mechanisms that can explain why the concentration of bacteria is normally so low in the small intestine while fairly high (up to 10^9/mL) in the terminal ileum, which is located at the end of the small intestine. These include the following: many ingested bacteria are destroyed in the stomach (via acid) and the proximal small intestine (via bile and enzymes); intestinal peristaltic activities significantly limit the amount of bacteria on the mucosal layer by forcefully transporting the bacteria forward (this mechanism, in addition to the ileocecal valve, prevents the bacteria from moving from the colon back to the ileum); and the immune system and secretory IgA (sIgA) play a role in selectively preventing the proliferation of certain bacteria. However, these mechanisms may function at suboptimal levels for multiple reasons, including: stress; a weakened immune system; lowered gastric acid—for instance, in those who regularly consume over-the-counter antacids, in the elderly, etc.; and irregular peristalsis—peristalsis can be affected in those who, for example, have stressful lifestyles or bad sleeping patterns. These irregular muscle movements may present with either decreased or increased bowel frequency, and/or a combination of both. These factors can increase the chance for a higher bacterial presence in the small intestine, which can cause digestive discomfort due to fermentation and gas formation, especially after ingestion of sugars, fiber, certain probiotics, and starch. If left unchecked, the altered environment of the small intestine can affect the absorption of essential nutrients, such as fats and proteins.

* This statement has not been evaluated by the Food and Drug Administration. This product is not intended to diagnose, treat, cure, or prevent any disease.

Apex Energetics™ products are intended for nutritional use and health maintenance only and should not replace or delay the use of any medication.

- Abdominal distention after consumption of fiber, starches, and sugar
- Abdominal distention after certain probiotic or natural supplements
- Decreased gastrointestinal motility, constipation
- Increased gastrointestinal motility, diarrhea
- Alternating constipation and diarrhea
- Suspicion of nutritional malabsorption
- Frequent use of antacid medication
- Diagnosed with celiac disease, irritable bowel syndrome, diverticulosis/diverticulitis, or leaky gut syndrome

Further Assessment of Category VII – Small Intestine

Further assessment is required in the following areas:

- Check for failure of brain-gut axis
- Check for gastric acid activity
- Check for history of acid blocking medications
- Check for gut motility:
  - Autonomic and enteric nervous system
- Check for alcohol consumption
- Check for intestinal inflammation:
  - Check for infections
  - Check for gluten sensitivity/celiac disease
  - Check for IBS
  - Check for Crohn's disease
- Intestinal Permeability
  - Cyrex® Laboratories Array 2
- Gluten Sensitivity/Crohn's Disease
  - Cyrex® Laboratories Array 3 Wheat/Gluten Proteome Reactivity & Autoimmunity
- IBS/Food Reactivity
  - Cyrex® Laboratories Array 10 Multiple Food Immune Reactivity Screen

MAF Blood Chemistry Key Association Category VII – Small Intestine

Usually only symptoms associated and not associated with blood chemistry markers. However, a CBC and stool exam may help recognize infections, anemia, etc.

MAF Nutritional Key Category VII – Small Intestine*

*Dietary and Nutritional Support for the Small Intestine:**

1. ClearVite-GL™◊ (K95), (K118): 1 scoop, with up to 4-6 ounces of water, once a day. However, a CBC and stool exam may help recognize infections, anemia, etc.
2. Sibiotica™◊ (K97): 1 capsule once a day.
3. RepairVite-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, once a day.
4. EnzymixPro™◊ (K99), (K99A): 1-2 capsules once a day.
5. EnteroVite™◊ (K100): 1 capsule 3 times a day.

   Alternative Formula
   - Tributyrin 350™◊ Active (K132): 1 softgel, 1-2 times a day, with a meal.
6. Dietary restrictions as directed by the Small Intestinal Microbial Environment Dietary and Nutritional Guide.
7. Exercises to increase motility as recommended by your healthcare professional.

Category VIII – Biliary Interpretation

Biliary insufficiency or stasis is a condition in which the body does not make enough bile or the bile is thick. Bile is a complex fluid consisting of various components including bile salts. It is secreted by the liver, and it is temporarily stored in the gallbladder to aid in fat digestion. The hormone cholecystokinin, released during eating, triggers the contraction of the gallbladder, which expels stored bile into the small intestine. In the small intestine, bile emulsifies fat, permitting its digestion by pancreatic enzymes.
The absorption of fat-soluble vitamins also requires bile emulsification. Biliary insufficiency or stasis can be caused by diets high in fat, elevated estrogen, or may be secondary to hypochlorhydria. Also note that if the individual has had his/her gallbladder removed, he/she will complain of biliary insufficiency symptoms because his/her cystic duct, which is very small, is now taking on the role of the gallbladder.

- Greasy or high-fat foods cause distress
- Lower bowel gas and/or bloating several hours after eating
- Bitter metallic taste in mouth, especially in the morning
- Burpy, fishy taste after consuming fish oils
- Unexplained itchy skin
- Yellowish cast to eyes
- Stool color alternates from clay colored to normal brown
- Reddened skin, especially palms
- Dry or flaky skin and/or hair
- History of gallbladder attacks or stones
- Gallbladder removed

Further Assessment of Category VIII – Biliary

Further assessment is required in the following areas:

- Check for high-fat diet with an excess of fried foods
- Check for failure of brain-gut axis
- Check for low thyroid

For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

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- Check for excess estrogens:
  - Exogenous (oral contraceptives, hormone replacement therapy, etc.)
  - Endogenous

MAF Blood Chemistry Key Association Category VIII – Biliary

- GGTP, alkaline phosphatase—don't usually show up until obstruction
- MAF symptoms may be due to thick bile, pre-gallstones, or gallstones
- Some individuals may have no symptoms because they avoid foods that cause them

MAF Nutritional Key Category VIII – Biliary*

*Nutritional Support for the Biliary System:**

1. BileMin™◊ (K11): 1-2 capsules with meals.
2. Lypomin-LV™◊ (K33): 1-3 capsules with meals.
   Alternative Formula
   - Metacrin-DX™◊ (K10): 1-2 capsules 2-3 times a day.
3. Super Methyl-SP™◊ (K112): 1-2 capsules twice a day.
   Alternative Formula
   - Methyl-SP™◊ (K14): 1-2 capsules with meals.

Additional Support Considerations:

1. C Liquazome™◊ (K139), (K139A): 1 teaspoon (4.93 mL or 1 packet) 1-2 times a day.
   Alternative Formula
   - C-Zyme 1000™◊ (Z4): 1 tablet with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
3. Mg/K-Zyme™◊ (Z11): 1 tablet with meals.

§Currently unavailable

Category IX – Hepatic Detoxication Interpretation

This category screens for symptoms related to hepatic detoxification integrity, more appropriately known as "hepatic biotransformation" or "hepatic detoxication." Many diverse symptoms may occur if the liver is inefficient in its abilities to convert compounds with phase I and phase II clearance pathways.

- Acne and unhealthy skin
- Excessive hair loss
- Overall sense of bloating
- Bodily swelling for no reason
- Hormone imbalances
- Weight gain
- Poor bowel function
- Excessively foul-smelling sweat

Further Assessment of Category IX – Hepatic Detoxication

Further assessment is required in the following areas:

- Check for excess alcohol, drugs, and environmental pollutants
- Check for hormone overload
- Check for poor diet

MAF Blood Chemistry Key Association Category IX – Hepatic Detoxication

- Usually only symptoms associated and not associated with blood chemistry markers
- May be suggested from functional elevation of SGOT, SGPT, and/or GGTP

MAF Nutritional Key Category IX – Hepatic Detoxication*

*Dietary and Nutritional Support for Hepatic Biotransformation:**

1. ClearVite™◊ Formula (K84, 95, 105, 108, 118, or 142): As directed by the ClearVite™◊ Basic Program Guide.
2. Hepato-Synergy™◊ (K69): 1 packet once a day or as needed.
3. Trizomal™◊ Glutathione (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.
4. Nourish Greens™◊ (K67): 1 scoop, with up to 8 ounces of water, once a day.

Category X – Sugar Metabolism Interpretation

(Also check for Hypoglycemia)

Reactive hypoglycemia refers to a state of low blood sugar, or states in which blood sugar levels spike up and down. Hypoglycemics suffer from fatigue and low energy because they do not supply their bodies with the amount and/or frequency of energy needed to support their metabolic system. Hypoglycemia is usually accompanied by adrenal exhaustion because the adrenals are called into play when

blood sugar levels spike or are low. The adrenals eventually become exhausted and need support. In individuals with hypoglycemia, dietary and lifestyle changes need to be made with regard to not skipping breakfast and snacking with low glycemic foods during the day.

- Crave sweets during the day
- Irritable if meals are missed
- Depend on coffee to keep going/get started
- Get light-headed if meals are missed
- Eating relieves fatigue
- Feel shaky, jittery, or have tremors
- Agitated, easily upset, nervous
- Poor memory, forgetful between meals
- Blurred vision

Further Assessment of Category X – Sugar Metabolism

Further assessment is required in the following areas:

- Check for diet:
  - Missing meals
  - Sugar, coffee, or fruit for meals
- Check for functional adrenal imbalance

MAF Blood Chemistry Key Association Category X – Sugar Metabolism

- Check for hypoglycemia:
  - Decreased glucose levels
  - Decreased LDH levels

MAF Nutritional Key Category X – Sugar Metabolism*

*Nutritional Support for Sugar Metabolism:**

1. Proglyco-SP™◊ (K13): 1 capsule 3 times a day, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
2. Butyric-FBR™◊ (K133): 2 capsules once a day.

   Alternative Formula
   - Fibromin™◊ (K25): 1 capsule, 2 times a day, or as needed.

Additional Support Considerations:

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 2 tablespoons once a day.§

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

Nutritional support for sugar metabolism should include:

Proglyco-SP™◊ (K13), which supplies the essential nutrients, minerals, and glandulars to support the adrenals and blood sugar metabolism; Adaptocrine™◊ (K2), (K124), which contains herbs that support sugar metabolism and the body's mental functionality under temporary stress; and Super EFA Complex™◊ (K8), which offers a source of essential fatty acids.*

Category XI – Peripheral Utilization of Sugars Interpretation

(Also check for Insulin Resistance)

Insulin resistance refers to a physiological pattern in which insulin receptor sites no longer respond to insulin. Insulin is a hormone that is secreted by the pancreas when serum glucose levels are elevated after meals. Insulin's job is to stimulate receptor sites, which will then transport glucose into the cells.

- Insulin resistance is caused by diets and physiological changes that cause prolonged and frequent release of insulin. Once insulin is repeatedly exposed to receptor sites, the sites will shut down and become nonresponsive. Insulin resistance will then lead to elevations of serum glucose because it cannot be carried into the cell, which leads to alterations in cholesterol and fat metabolism. Insulin resistance places great demands on the beta cells of the pancreas for insulin output, and may eventually lead to diabetes if this is not addressed properly.
- Fatigue after meals
- Crave sweets during the day
- Eating sweets does not relieve cravings for sugar
- Must have sweets after meals
- Waist girth is equal or larger than hip girth
- Frequent urination
- Increased thirst and appetite
- Difficulty losing weight

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

Further Assessment of Category XI – Peripheral Utilization of Sugars

Further assessment is required in the following areas:

- Check for diet:
  - Excess-starch diet
  - Overeating
- Check for elevated cortisol response
- Check for autoimmune issue:
  - Check for late-onset autoimmune diabetes
    - GAD antibodies, islet cell antibodies

MAF Blood Chemistry Key Association Category XI – Peripheral Utilization of Sugars*

- Check for insulin resistance:
  - Increased glucose levels
  - Increased triglycerides levels
  - Increased total cholesterol and LDL levels
  - Decreased HDL levels
  - Increased uric acid levels

MAF Nutritional Key Category XI – Peripheral Utilization of Sugars

*Nutritional Support for the Peripheral Utilization of Sugars:**

1. Glysen™◊ (K1), (K135): 1-2 capsules with meals, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
2. Glycoberine-MX™◊ (K126): 1 capsule with a meal.
   Alternative Formula
   - Glycoberine™◊ Active (K127): 1 capsule with a meal.
3. EnteroVite™◊ (K100): 1-2 capsules with meals.
   Alternative Formula
   - Butyric-FBR™◊ (K133): 1-2 capsules with meals.

Additional Support Considerations:

1. Glysen-Synergy™◊ (K79): 1 packet, once a day, or as needed.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
   Alternative Formula
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
3. Resvero Active™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) 2 times a day.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.
2. Oxicell™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.
   Alternative Formula
   Super Oxicell™◊ (KR23): Apply in the morning, afternoon, and evening.

To provide comprehensive nutritional support for the peripheral utilization of sugars, Glysen-Synergy™◊ (K79) combines four complementary formulas Glysen™◊ (K1), (K135); Adaptocrine™◊ (K2), (K124); Fibromin™◊ (K25); and Protoglysen™◊ (K28) into one easy-to-use package. OmegaCo3™◊ (K7) is a source of essential fatty acids.*

Furthermore, adaptogens, which are herbs that support metabolism of sugar's adrenal feedback loop coordination, such as Adaptocrine™◊ (K2), (K124), are included.* A source of nonsoluble fibers, such as Fibromin™◊ (K25), is included to support gastrointestinal glucose uptake.*

Category XII – Adrenal Interpretation

(Also check for Adrenal Hypofunction)

The adrenal glands are two small glands that sit on top of the kidneys. They produce several important hormones: cortisol to regulate glucose levels, aldosterone to regulate sodium and potassium levels, DHEA to modulate the immune system, and catecholamines to excite the sympathetic nervous system. The adrenal glands are called the anti-stress glands because they allow the body to deal with stress. Stress can be mental/emotional from unhealthy relationships, chemical from inadequate diets (reactive hypoglycemia and food sensitivities), or physical from overtraining. Individuals who present with adrenal exhaustion must avoid caffeine, alcohol, and sugar, as well as any foods to which they may be sensitive.

- Cannot stay asleep
- Crave salt
- Slow starter in the morning
- Afternoon fatigue
- Dizziness when standing up quickly
- Afternoon headaches
- Headaches with exertion or stress
- Weak nails

Further Assessment of Category XII – Adrenal

Further assessment is required in the following areas:

- Check for hypoglycemia lifestyle
- Check for impaired adrenals from:
  - Chronic infection
  - Environmental insult

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- Autoimmunity
- Food sensitivities
- Intestinal permeability
- High-stress lifestyle
- Overtraining

MAF Blood Chemistry Key Association Category XII – Adrenal

- Check for hypofunction:
  - Sodium and potassium changes
- Check Category X on Metabolic Assessment Form™ results to assess sugar metabolism

MAF Nutritional Key Category XII – Adrenal*

*Nutritional Support for Adrenal Function:**

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

*May Also Need to Support Sugar Metabolism (see Category X):**

1. Proglyco-SP™◊ (K13): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

To support adrenal function and sugar metabolism, consider using Proglyco-SP™◊ (K13).* Adaptocrine™◊ (K2), (K124), which is a multi-use herbal formula designed to support a hypothalamic-pituitary-adrenal (HPA) feedback loop should also be considered.* Super EFA Complex™◊ (K8) can be used as a source of essential fatty acids.*

Category XIII – Adrenal* Interpretation

(Also check for Adrenal Hyperfunction)

Adrenal hyperfunction is the initial adrenal response to stress in which the adrenals will increase their release of cortisol. This response is called the alarm reaction and can last for years. Adrenal hyperfunction has the tendency to induce insulin sensitivity and vice versa.

- Cannot fall asleep
- Perspire easily
- Under a high amount of stress
- Weight gain when under stress
- Wake up tired even after 6 or more hours of sleep
- Excessive perspiration or perspiration with little or no activity

Further Assessment of Category XIII – Adrenal

Further assessment is required in the following areas:

- Check for hyperglycemia lifestyle
- Check for impaired adrenals from:
  - Chronic infection
  - Environmental insult
  - Autoimmunity
  - Food sensitivities
  - Intestinal permeability
  - High-stress lifestyle
  - Overtraining

MAF Blood Chemistry Key Association Category XIII – Adrenal

- Check for hyperfunction:
  - Sodium and potassium changes
- Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars

MAF Nutritional Key Category XIII – Adrenal*

*Nutritional Support for Adrenal Function:**

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

*May Also Need to Support the Peripheral Utilization of Sugars (see Category XI):**

1. Glysen™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
2. OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.

   Alternative Formula
   - OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

MAF Nutritional Key
Category XIII – Adrenal* (Cont.)

Additional Formula

- Neurosera™◊ (K136): 1-2 teaspoons (5-10 mL) once a day.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

Nutritional support for adrenal function should include Adaptocrine™◊ (K2), (K124), which is a multi-use adrenal formula used to support the HPA feedback control; and Glysen™◊ (K1), (K135), and OmegaCo3™◊ (K7), (K113), which are used to support the peripheral utilization of sugars.*

Category XIV – Electrolyte and pH Balance Interpretation

This section of the manual is related to electrolyte and pH balance. Imbalances may occur secondary to diet, exercise, hydration, renal imbalances, adrenal imbalances, dysglycemia, and pulmonary imbalances. Careful screening for related mechanisms should be conducted if symptoms are present.

- Edema and swelling in ankles and wrists
- Muscle cramping
- Poor muscle endurance
- Frequent urination
- Frequent thirst
- Crave salt
- Abnormal sweating from minimal activity
- Alteration in bowel regularity
- Inability to hold breath for long periods
- Shallow, rapid breathing

Further Assessment of Category XIV – Electrolyte and pH Balance

Further assessment is required in the following areas:

- Check for tired/sluggish
- Check for cold—hands, feet, all over
- Check for excess acidifying diet
- Check for overhydration
- Check for physiological failure in pH buffering system
- Check for physiological imbalance in metabolism of sodium, potassium, chloride, calcium, or magnesium

MAF Blood Chemistry Key Association
Category XIV – Electrolyte and pH Balance

- Check for sodium, potassium, chloride, calcium, and magnesium levels
- Check for CO_2 levels
- Check for anion gap

MAF Nutritional Key
Category XIV – Electrolyte and pH Balance*

*Nutritional Support for Electrolyte and pH Balance:**

1. Electro-pH™◊ Complex (K86): 1 scoop with up to 8 ounces of water.

Category XV – Thyroid Interpretation

(Also check for Hypofunction)

The thyroid gland is in charge of two main functions. First, it controls the rate of metabolism in the body, and second, it helps regulate calcium levels in the bone through the hormone calcitonin. The symptoms of low thyroid function may originate from numerous alterations in metabolism, such as a decrease in thyroid or pituitary hormone synthesis, altered hypothalamic-pituitary-feedback coordination, altered thyroid hormone transport, altered peripheral hormone metabolism, hormone detoxification, thyroid receptor-site resistance, etc.

- Tired/sluggish
- Feel cold—hands, feet, all over
- Require excessive amounts of sleep to function properly
- Increase in weight even with low-calorie diet
- Gain weight easily
- Difficult, infrequent bowel movements
- Depression/lack of motivation
- Morning headaches that wear off as the day progresses
- Outer third of eyebrow thins
- Thinning of hair on scalp, face, or genitals, or excessive hair loss
- Dryness of skin and/or scalp
- Mental sluggishness

Further Assessment of Category XV – Thyroid

Further assessment is required in the following areas:

- Check for Hashimoto's response
- Check for goitrogenic impairment
- Check for T4-to-T3 conversion impairment

- Check for pituitary TSH release impairment
- Check for thyroid-binding globulin imbalance
- Check for metabolic syndrome

MAF Blood Chemistry Key Association Category XV – Thyroid

- Check for hypofunction:
  - Increased TSH levels
  - T3 uptake
  - Total T4 levels
  - Total T3 levels
  - FTI
  - Increased thyroid antibodies

MAF Nutritional Key Category XV – Thyroid*

*Nutritional Support for Thyroid Function:**

1. Thyroxal™◊ (K12): 1-2 capsules with meals.

*Nutritional Support for Thyroid Immune Pathways:**

1. Thyro-FLM™◊ (K137): 2 capsules once a day.

*Nutritional Support for the Thyroid:**

1. Thyro-CNV™◊ (K9): 1-2 capsules with meals.

*Nutritional Support for T3:**

1. Thyro-CNV™◊ (K9): 1-2 capsules with meals.
2. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

Category XVI – Thyroid Interpretation

(Also check for Hyperfunction)

There are different degrees of overactive thyroid function. Some may be managed functionally, and some may need the intervention of medications and hormones for a brief period of time. The main culprit of an overactive thyroid includes some type of antigen that triggers an inflammatory or autoimmune response. The antigen may be a virus, a food sensitivity, a gut pathogen that has altered immune responses, etc.

- Heart palpitations
- Inward trembling
- Increased pulse even at rest
- Nervous and emotional
- Insomnia
- Night sweats
- Difficulty gaining weight

Further Assessment of Category XVI – Thyroid

Further assessment is required in the following areas:

- Check for Hashimoto's response
- Check for excessive thyroid hormone intake
- Check for neurochemical imbalance
- Check for autoimmune issue:
  - GAD antibodies

MAF Blood Chemistry Key Association Category XVI – Thyroid*

- Check for hyperfunction:
  - Decreased TSH levels
  - T3 uptake
  - Total T4 levels
  - Total T3 levels
  - FTI
  - Increased thyroid antibodies

MAF Nutritional Key Category XVI – Thyroid*

*Nutritional Support for the Thyroid:**

1. Trizomal™◊ Glutathione (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.
   Alternative Formulas
   - AC-Glutathione 250™◊ (K117): 1 capsule 1-2 times a day.
   - AC-Glutathione™◊ (K88): 1-2 capsules 1-2 times a day.
2. ClearVite™◊ Formula (K84, 95, 105, 108, 118, or 142): As directed by the ClearVite™◊ Basic Program Guide.
3. Nitric Balance™◊ (K62), (K68), (K82): 1 teaspoon once a day.

Additional Support Considerations:

1. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
2. Ultra-D 5000™◊ (K78): 1-4 teaspoons, once a day, with meals.

For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

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MAF Nutritional Key Category XVI – Thyroid* (Cont.)

Alternative Formulas

- Ultra-D Complex™◊ (K35): 1-4 teaspoons, once a day, with meals.
- AD-Pro™◊ (K119): 1 softgel once a day.
- ADEK-Pro™◊ (K102): 1-3 softgels once a day.

For additional vitamin D, consider Liqua-D™◊ (K87): 1 drop once a day.

§Currently unavailable

Also Consider:

Oxicell™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.

Alternative Formula

- Super Oxicell™◊ (KR23): Apply in the morning, afternoon, and evening.

There are three products to naturally support the thyroid: Thyroxal™◊ (K12), Thyro-CNV™◊ (K9), and AC-Glutathione™◊ (K88), (K117).* Thyroxal™◊ (K12) is a multi-use nutritional, mineral, herbal, and glandular support for thyroid function that does not contain iodine or tyrosine.* Additional iodine support for the thyroid is contraindicated in individuals who consume a high-salt diet. The amino acid tyrosine is contraindicated in individuals suffering from a stress response. Thyro-CNV™◊ (K9) is a multi-use vitamin, mineral, and herbal product designed to support the conversion of peripheral T4 to T3.* AC-Glutathione™◊ (K88), (K117) offers 125 mg per capsule of S-acetyl L-glutathione, which is an alternative form of reduced glutathione.

Category XVII – Prostate Interpretation (Males Only)

The prostate is a small, chestnut-shaped gland that lies below the bladder and surrounds the urethra. Nonmalignant enlargement of the prostate gland is called benign prostatic hypertrophy (BPH) and is characterized by symptoms of bladder obstruction, such as increased urinary frequency, nighttime awakenings to empty the bladder, and reduced force and caliber (speed of flow) of urination. The key issue with BPH is to differentiate BPH from the more serious prostate cancer. If the clinician is suspicious of malignancy, follow-up diagnostic evaluations, such as the manual prostate exam, a prostate-specific antigen (PSA) and free PSA test, and even an ultrasound to make the differentiation by a qualified urologist should be performed.

- Urination difficulty or dribbling
- Frequent urination
- Pain inside of legs or heels
- Feeling of incomplete bowel emptying
- Leg twitching at night

Further Assessment of Category XVII – Prostate

Further assessment is required in the following areas:

- Clinician needs to consider the possibilities of benign prostatic hyperplasia or prostate cancer so check for the following:
  - Prostate-specific antigen (PSA)
  - Free PSA
  - Imaging

MAF Blood Chemistry Key Association Category XVII – Prostate

- Check for PSA levels
- Check for free PSA levels

MAF Nutritional Key Category XVII – Prostate*

*Nutritional Support for the Prostate:**

1. Prosta-DHT™◊ (K6): 1-2 capsules with meals.

Additional Support Considerations:

1. Fibromin™◊ (K25): 1-3 capsules with meals.
2. Estrovite™◊ (K5): 1-2 capsules with meals.
3. Bladder Serene™◊ (K138): 1 scoop (2.6 g), with up to 6 ounces of water, once a day.
4. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas

   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

To provide nutritional support for the prostate, consider using Prosta-DHT™◊ (K6), Super EFA Complex™◊ (K8), and Estrovite™◊ (K5).* Prosta-DHT™◊ (K6) is a multi-use nutritional, herbal, and amino acid complex to support testosterone levels.* Super EFA Complex™◊ (K8) offers a wide variety of essential fatty acids required to support the prostate.* DHT, which is responsible for prostate tissue hyperplasia, is unable to be excreted from the prostate and therefore contributes to hyperplasia with elevated estrogen levels. Estrovite™◊ (K5) is a multi-use nutritional and herbal formula to support estrogen metabolism and detoxification.*

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A source of supplemental fiber to support the amount of sex hormone-binding globulin can be found in Fibromin™◊ (K25).*

Category XVIII – Andropause (Males Only) Interpretation

This category is a quick screen for the typical symptoms of andropause. These symptoms take place when there are low androgen levels, such as testosterone, DHT, or androstenedione. These symptoms can also take place if there is a low testosterone-to-estrogen ratio. This ratio manifests when there is a state of estrogen dominance. If the individual has numerous symptoms in this category, andropause should be considered. There are many factors and systems involved in the patterns associated with andropause. These factors include: liver detoxification, estrogen metabolism, gastrointestinal microflora activity, insulin sensitivity, adrenal function, testosterone synthesis, 5-alpha reductase activity, beta-glucuronidase activity, 17,20-lyase activity, aromatase activity, and the feedback loops involved with the pituitary and the hypothalamus. A male hormone profile that includes measurements of testosterone, androstenedione, DHEA, progesterone, estrone, LH, and FSH can be a very useful and accurate tool to identify the patterns of andropause in addition to the andropause questionnaire.

- Decreased libido
- Decreased number of spontaneous morning erections
- Decreased fullness of erections
- Difficulty maintaining morning erections
- Spells of mental fatigue
- Inability to concentrate
- Episodes of depression
- Muscle soreness
- Decreased physical stamina
- Unexplained weight gain
- Increase in fat distribution around chest and hips
- Sweating attacks
- More emotional than in the past

Further Assessment of Category XVIII – Andropause (Males Only)

Further assessment is required in the following areas:

- Check for insulin surge promotion of aromatase
- Check for inflammatory compromise of Leydig cells
- Check for stress-response suppression of LH
- Check for Dihydrotestosterone (DHT) elevation, leading to DHT receptor-site resistance

MAF Blood Chemistry Key Association Category XVIII – Andropause (Males Only)

- Check for insulin resistance
- Check for systemic inflammation
- Check for GI infections
- Check for neurodegeneration
- Check hormone levels (testosterone:estrogen ratio)

MAF Nutritional Key Category XVIII – Andropause* (Males Only)

*Nutritional Support for Testosterone Levels:**

1. Opticrine™◊ (K3): 1-2 capsules with meals.
2. DHEA-25C™◊ (K141): 1 capsule once a day.
   Alternative Formula
   - DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

*Nutritional Support for Estrogen Levels:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.
2. Glysen™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
3. OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   Alternative Formula
   - OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
4. Fibromin™◊ (K25): 1 capsule, 2 times a day, or as needed.
5. Resvero Active™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) once a day.
   Alternative Formula
   - Resvero-SE™◊ Active (K125): 1 softgel once a day.

Also Consider:

1. Testanex™◊ (KR17), (KR74): Apply 2 times a day.

Many nutritional supplements can be effective in supporting a male hormone balance.* For example, Opticrine™◊ (K3) can be used to support testosterone synthesis.* The peripheral utilization of sugars can contribute to excess estrogen levels by upregulating aromatase. Glysen™◊ (K1) and OmegaCo3™◊ (K7), (K113) are essential supports for the peripheral utilization of sugars.* A source of supplemental fiber to support sex hormone-binding globulin levels can be found in Fibromin™◊ (K25).*

Category XIX – Menstruating Women Interpretation

The female reproductive cycle is very sensitive, even to slight alterations in hormone metabolism. Alterations in

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

menstruation should give the clinician clues as to which systems may need support.

- Perimenopausal
- Alternating menstrual cycle lengths
- Extended menstrual cycle (greater than 32 days)
- Shortened menstrual cycle (less than 24 days)
- Pain and cramping during periods
- Scanty blood flow
- Heavy blood flow
- Breast pain and swelling during menses
- Pelvic pain during menses
- Irritable and depressed during menses
- Acne
- Facial hair growth
- Hair loss/thinning

Further Assessment of Category XIX – Menstruating Women

Further assessment is required in the following areas:

- Check for blood glucose issues
- Check for liver detoxification need
- Check for essential fatty acid imbalances

MAF Blood Chemistry Key Association Category XIX – Menstruating Women

- Check for systemic inflammation
- Check hormone levels

MAF Nutritional Key Category XIX – Menstruating Women*

*Nutritional Support for Progesterone Function:**

1. Progestaid™◊ (K4): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.

*Nutritional Support for Adrenal Function:**

1. Proglyco-SP™◊ (K13): 1-2 capsules with meals.
2. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.
3. OmegaCo3-SE™◊ (K113): 1-2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1-2 tablespoons 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

Progestaid™◊ (K4) is a natural herbal and nutritional product to support luteal function and progesterone output.*
Estrovite™◊ (K5) is a multi-use vitamin, mineral, and herbal product to support estrogen receptor activity, detoxification of estrogens, and estrogen metabolism.*
Super EFA Complex™◊ (K8) offers the female a variety of essential fatty acids to support female hormone metabolism.* To support adrenal function and hormone production, women can use Adaptocrine™◊ (K2), (K124), and Proglyco-SP™◊ (K13).*

Category XX – Menopausal Women Interpretation

The symptoms of menopause can be related to numerous functional imbalances. A comprehensive female hormone profile may be required for specific nutritional protocols. However, some nutritional products may prove to be helpful in most cases.

- How many years menopausal
- Since menopause any uterine bleeding
- Hot flashes
- Mental fogginess
- Disinterest in sex
- Mood swings
- Depression
- Painful intercourse
- Shrinking breasts
- Facial hair growth
- Acne
- Increased vaginal pain, dryness, or itching

Further Assessment of Category XX – Menopausal Women

Further assessment is required in the following areas:

- Check for blood glucose issues
- Check for liver detoxification need
- Check for essential fatty acid imbalances

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MAF Blood Chemistry Key Association Category XX – Menopausal Women

- Check for systemic inflammation
- Check hormone levels

MAF Nutritional Key Category XX – Menopausal Women*

*Nutritional Support for Progesterone Function:**

1. Progestaid™◊ (K4): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
3. DHEA-25C™◊ (K141): 1 capsule once a day.

   Alternative Formula
   - DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

§Currently unavailable

*Nutritional Support for Progesterone Function:**

1. Pregnenolone (K19): 10 drops (0.255 mL) 1-3 times a day.

Progestaid™◊ (K4) is a natural herbal and nutritional product to support luteal function and progesterone output.* Estrovite™◊ (K5) is a multi-use vitamin, mineral, and herbal product to support estrogen receptor activity, support a detoxification of estrogens, and support estrogen metabolism.* Super EFA Complex™◊ (K8) offers a variety of essential fatty acids to support female hormone metabolism.*

Part III: Foods

This category screens for foods that have both a positive and negative impact on metabolism, such as essential fatty acids, lignans, dietary fiber, caffeine, and partially hydrogenated fats. The individual's consumption of raw nuts, seeds, and fish will give the clinician a general idea of the amount of essential fatty acids (EFAs) in his/her diet. The clinician needs to clarify that toasted or flavored nuts, canned tuna, and fried fish do not count as a source of EFAs. If the individual has a low dietary intake of EFAs, an EFA supplement should be recommended. Most individuals will require an omega-3 supplement versus an omega-6 source.

The individual's consumption of vegetables needs to be identified. Vegetables are necessary to provide a source of fiber and lignans that are crucial in managing metabolism. These substances have a positive impact on the gastrointestinal tract and its influence on hormone metabolism. This is especially true in elevated hormone patterns. The source of dietary fibers from plants has the potential to inhibit the activity of gastrointestinal beta-glucuronidase, which deconjugates estrogens conjugated in the liver.

The enzyme beta-glucuronidase is found in the large intestine and has the potential to allow hormones to be reabsorbed and remetabolized if the diet is low in fiber. Cruciferous vegetables also contain a chemical component called indole-3-carbinol, which can promote C-2 hydroxylation over C-4 and/or C-16 hydroxylation of estrogens. This will shift estrogen metabolism ratios to 2-hydroxyestrone, which has low estrogenic activity compared to 4-hydroxyestrone or 16 alpha-hydroxyestrone, which has powerful estrogenic activity. Fiber and lignans in the diet also increase the circulating concentrations of sex hormone-binding globulin (SHBG), thus reducing levels of unbound, active hormones.

The consumption of caffeine can have an adverse impact on metabolism at multiple levels. First, caffeine is an adrenal stimulant and can promote overactivity and shift pregnenolone away from androgen synthesis into glucocorticoid synthesis. Second, caffeine increases the demand placed on hepatic detoxification and can deplete the substrates required for hormone detoxification by the cytochrome P450 family. Third, caffeine can have an adverse impact on gastrointestinal health. If an individual suffers from metabolic alterations and is not willing to stop his/her intake of caffeine, he/she will respond very slowly. Please note that decaffeinated coffee still contains 60% caffeine, and is not an acceptable alternative. It is important to educate individuals that their dependence on caffeine may be resolved if they support their adrenal glands and stabilize their glucose levels during the day.

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

Fast food consumption on a regular basis can make handling metabolic issues very difficult. Fast food can have an adverse impact on all the major systems needed to promote optimal hormone function. Most fast foods are high in trans-fatty acids and will have a negative impact on EFA metabolism and gastrointestinal health. Educating the individual to choose foods such as salads (without dressings that contain partially hydrogenated oils), quality fish, and unprocessed natural foods can have a profound impact on correcting altered metabolic patterns.

The individual's classification of the three healthiest and three worst foods eaten during an average week is a quick and easy way to understand how he/she views the quality of foods in his/her diet. Many times, individuals are uninformed about healthy foods.

The identification of the individual's stress level is also important to understand. Management of stress can be a powerful tool in establishing good health and function. Ask the patient:

- How many alcoholic beverages do you consume per week?
- How many caffeinated beverages do you consume per day?
- How many times do you eat out per week?
- How many times do you eat raw nuts or seeds per week?
- How many times do you eat fish per week?
- How many times do you work out per week?
- List the three worst foods you eat during the average week.
- List the three healthiest foods you eat during the average week.
- Rate your stress level on a scale of 1-10 during the average week.

Part IV: Medications

This category of the questionnaire allows the clinician to screen the individual's medication intake. It gives the clinician clues to which systems are imbalanced. In addition, it allows the clinician to consider any drug-nutrient interactions.

- List any medications currently taking and for what conditions
- List any supplements currently taking and reason

Antacids

Individuals taking antacids need to be screened for hypochlorhydria; *H. pylori* inoculation; parasitic inoculation; gastrointestinal IgA-mediated food sensitivities; hypothalamus and/or pituitary hypofunction, which alters the release of gastrin; and reactive hypoglycemia, which causes the depletion of essential cofactors for HCl production. It should be noted that when people take antacids and alkalinize their gastrointestinal tract, the absorption of essential nutrients such as zinc, calcium, vitamin B_{12}, and iron may be compromised. In addition to alterations in nutrient absorption, the alkalinizing effect of antacids also may alter proper digestion by compromising the release of cholecystokinin. The end result of antacid consumption may be improper digestion of food due to an inhibitory release of hydrochloric acid, pancreatic enzymes, and bile.

Antibiotics

Individuals taking antibiotics need to be screened for healthy gastrointestinal immunity and dysbiosis. Supporting the body with beneficial bacteria should always be considered.

Antifungals

Individuals taking antifungals need to be screened for alterations in healthy immunity. Antifungals inhibit the steroid hormone enzyme 17,20-lyase, which will shut down the synthesis of DHEA, estrogen, and testosterone.

Antihistamines

Individuals that require antihistamines need to be screened for food sensitivities, hypochlorhydria, adrenal function, and EFA metabolism function.

Antidepressants

Individuals taking antidepressants need to be screened for thyroid imbalances, adrenal insufficiency, blood sugar disorders, and the need for serotonin cofactors, such as 5-HT, ascorbic acid, vitamin B_5, folic acid, iron, and niacin.

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Insulin Management Agents

Individuals taking medications to support proper clearance of glucose or diabetics who require insulin need to be screened for adrenal cortical hyperfunction. The management of physiological inflammation and the proper intake of EFA in the diet need to be considered.

NSAIDs and Anti-Inflammatory Agents

Individuals taking anti-inflammatory agents need to be screened for proper adrenal function, EFA metabolic defects, zinc need, magnesium need, or a need for vitamin B_6.

Anxiety Medications

Individuals taking anxiety medication need to be screened for proper hypothalamus-pituitary-adrenal function, B vitamin status, gastric reflux, hypochlorhydria, and proper breakdown of catecholamines by methylation.

Blood Pressure-Lowering Agents

Individuals taking high blood pressure medications should be screened for adrenal gland hyperfunction, gastrointestinal dysfunction, food sensitivities, EFA deficiencies, magnesium need, etc. High blood pressure medications are categorized into diuretics, ACE inhibitors, beta blockers, and calcium channel blockers.

Cholesterol-Lowering Agents

Individuals with high cholesterol must be ruled out for hypothyroidism, dysinsulinism, gastrointestinal imbalances, and biliary stasis. Cholesterol-lowering drugs can be divided into three main categories: HMG CoA reductase inhibitors, bile acid sequestering resins, and fibric acid derivatives.

Oral Contraceptives

Individuals taking oral contraceptives need to be evaluated for thyroid underconversion and estrogen dominance.

Hormones

Individuals taking hormones should have liver and biliary support to promote a healthy hormone balance.

Laxatives

Individuals taking laxatives need to drink adequate water, exercise, and have adequate fiber in their diet. In addition, the clinician must rule out thyroid hypofunction, hypochlorhydria, biliary stasis, dysbiosis, and food sensitivities.

Diuretics

Individuals taking diuretics are given the medication to remove water from the body. They are used to lower blood pressure in people with hypertension and to assist the heart to pump more efficiently in people with congestive heart failure. Diuretics are also used to reduce water accumulation caused by other diseases. Diuretics will induce thiamine and magnesium deficiency. All individuals taking diuretics should be considered for a source of thiamine and a source of magnesium. A broad-spectrum cardiovascular supplement should also be considered.

Cortisone

Individuals taking cortisone for inflammatory conditions need to have EFA imbalances and adrenal insufficiency ruled out. Individuals taking corticosteroids for autoimmune conditions need to be screened for allergies, sensitivities, and gastrointestinal imbalances. Cortisone will inhibit the body's HPA axis and suppress the body's natural ability to produce glucocorticoids. Cortisone will suppress humoral immunity and IgA cell production and will induce deficiencies of magnesium, melatonin, potassium, vitamin B_6, vitamin C, vitamin D, vitamin K, vitamin A, and zinc. Individuals taking cortisone-containing agents, such as pills, injections, or creams, will induce false elevations of cortisol if an adrenal salivary index is conducted.

The lab tests and blood markers referenced on this form are solely intended for nutritional evaluation within the reference range. Blood test markers that are out of range may require pharmaceutical intervention or other medical treatment and will require that the individual be seen by a medical practitioner with a scope of practice to diagnose, treat, and cure disease.

Symptom groups listed here and on the MAF are not intended to be used as a diagnosis of any disease or condition. For medical evaluation of symptoms, especially frequent or severe symptoms, please seek appropriate conventional medical management by a physician. For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

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Metabolic Assessment Form Nutritional Key

CATEGORY I: Colon

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Colon:**

4-R Program: Remove, Replace, Reinoculate, Revive

Remove: GI-SYNERGY™◊ (K64), (K83): 1 packet once a day.

Replace: BILEMIN™◊ (K11): 1-2 capsules with meals.
ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Reinoculate: STRENGTIA™◊ (K61): 1-3 capsules a day.

Alternative Formula
- SIBIOTICA™◊ (K97): 1-3 capsules twice a day.

Revive: REPAIRVITE™◊ (K60), (K63): 1 scoop, with up to 8 ounces of water, 1-2 times a day.

Alternative Formulas
- REPAIRVITE-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, twice a day.
- REPAIRVITE-GT™◊ PLUS (K120): 1 scoop, with up to 8 ounces of water, once a day.

STRENGTIA™◊ (K61): 1-3 capsules a day.

Alternative Formula
- SIBIOTICA™◊ (K97): 1-3 capsules twice a day.

Please consider the Mucosal Health & Food Tolerance Program (from the corresponding guide).

CATEGORY II: Intestinal Barrier

(also check for Intestinal Permeability)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Intestinal Barrier:**

REPAIRVITE™◊ (K60), (K63): 1 scoop, with up to 8 ounces of water, 1-2 times a day.

Alternative Formulas
- REPAIRVITE-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, once a day.
- REPAIRVITE-GT™◊ Plus (K120): 1 scoop, with up to 8 ounces of water, once a day.

ENTEROVITE™◊ (K100): 1 capsule 3 times a day.

TRIZOMAL™◊ GLUTATHIONE (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.

Additional Support Considerations:

GI-SYNERGY™◊ (K64), (K83): 1 packet, twice a day, with a meal.

STRENGTIA™◊ (K61): 1-3 capsules a day.

Alternative Formula
- SIBIOTICA™◊ (K97): 1-3 capsules twice a day.

TURMERO™◊ ACTIVE (K75), (K109): 5 mL (approx. 1 teaspoon) 1-2 times a day.

RESVERO™◊ ACTIVE (K76), (K110): 5 mL (approx. 1 teaspoon) once a day.

AD-PRO™◊ (K119): 1 softgel, once a day, with a meal.

Alternative Formulas
- ADEK-PRO™◊ (K102): 1-3 softgels once a day.
- ULTRA-D 5000™◊ (K78): 1-4 teaspoons, once a day, with meals.
- ULTRA-D COMPLEX™◊ (K35): 1-4 teaspoons, once a day, with meals.

For additional vitamin D, consider LIQUA-D™◊ (K87): 1 drop once a day.

Please consider the RepairVite™◊ Program (from the corresponding guide).

CATEGORY III: Chemical Tolerance

(also check for Loss of Chemical Tolerance)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Dietary and Nutritional Support for the Immune System:**

REPAIR & CLEAR™◊ Program (basic or comprehensive):
As directed by the REPAIR & CLEAR™◊ Program Guide.

CATEGORY IV: Stomach

(also check for Hypochlorhydria)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Healthy Stomach Acid Levels and the Gastrointestinal System:**

ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Alternative Formulas
- HCL PRO-ZYME™◊ (Z26): 1-2 tablets with each meal.
- SUPERDIGEST-ZYME™◊ (Z22): 1-2 tablets with each meal.

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

Immune Support During *H. pylori*
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for pH Balance and the Gastrointestinal System:**

H-PLR™◊ (K32): 1-2 capsules with each meal.

ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Alternative Formulas
- HCL PRO-ZYME™◊ (Z26): 1-2 tablets with each meal.
- SUPERDIGEST-ZYME™◊ (Z22): 1-2 tablets with each meal.

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

CATEGORY V: Stomach

(also check for Hyperacidity)
Hyperacidity Secondary to Hypochlorhydria
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for pH Balance, Healthy Stomach Acid Levels, and the Gastrointestinal System:**

ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Alternative Formulas
- HCL PRO-ZYME™◊ (Z26): 1-2 tablets with each meal.
- SUPERDIGEST-ZYME™◊ (Z22): 1-2 tablets with each meal.

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

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CATEGORY V: Stomach (Continued)

Hyperacidity with Ulcer or Preulcerous State
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Gastrointestinal Health:**

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.
H-PLR™◊ (K32): 1-2 capsules with each meal.

Hyperacidity with *H. pylori* Inoculation
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for pH Balance and the Gastrointestinal System:**

H-PLR™◊ (K32): 1-2 capsules with each meal.
ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Alternative Formulas

- HCL PRO-ZYME™◊ (Z26): 1-2 tablets with each meal.
- SUPERDIGEST-ZYME™◊ (Z22): 1-2 tablets with each meal.

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.
MYCO-ZYME™◊ (Z13): 1-2 capsules with meals.

Hyperacidity Due to Stress
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for pH Balance and the Adrenals:**

GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

Also Consider:

ADRENACALM™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

CATEGORY VI: Pancreas

Pancreatic Insufficiency
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Pancreas:**

ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.
GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

Pancreatic Insufficiency Secondary to Hypochlorhydria
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Pancreas and Healthy Stomach Acid Levels:**

ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.

Alternative Formula

- HCL PRO-ZYME™◊ (Z26): 1-2 tablets with meals.

Additional Formula

- SUPERDIGEST-ZYME™◊ (Z22): 1-2 tablets with meals.

CATEGORY VI: Pancreas (Continued)

Small Intestine Dysfunction
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Dietary and Nutritional Support for the Small Intestine:**

CLEARVITE™◊ Basic Program: Three-week intestinal support program as directed by the CLEARVITE™◊ Basic Program Guide.
GASTRO-ULC™◊ (K29): 1 tablet, 15-20 minutes before a meal, three times a day.

CATEGORY VII: Small Intestine

Small Intestinal Bacterial Overgrowth
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Dietary and Nutritional Support for the Small Intestine:**

CLEARVITE-GL™◊ (K95), (K118): 1 scoop, with up to 4-6 ounces of water, once a day.
SIBIOTICA™◊ (K97): 1 capsule once a day.
REPAIRVITE-SE™◊ (K98): 1 scoop, with up to 4-6 ounces of water, once a day.
ENZYMIXPRO™◊ (K99), (K99A): 1-2 capsules with each meal.
ENTEROVITE™◊ (K100): 1 capsule 3 times a day.

Alternative Formula

- TRIBUTYRIN 350™◊ ACTIVE (K132): 1 softgel, 1-2 times a day, with a meal.

Dietary restrictions as directed by the Small Intestinal Microbial Environment Dietary and Nutritional Guide.

Exercises to increase motility as recommended by your healthcare professional.

CATEGORY VIII: Biliary

Biliary Insufficiency and/or Stasis
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Biliary System:**

BILEMIN™◊ (K11): 1-2 capsules with meals.
LYPOMIN-LV™◊ (K33): 1-3 capsules with meals.

Alternative Formula

- METACRIN-DX™◊ (K10): 1-2 capsules 2-3 times a day.

SUPER METHYL-SP™◊ (K112): 1-2 capsules twice a day.

Alternative Formula

- METHYL-SP™◊ (K14): 1-2 capsules with meals.

Additional Support Considerations:

C LIQUAZOME™◊ (K139), (K139A): 1 teaspoon (4.93 mL or 1 packet) 1-2 times a day.

Alternative Formula

- C-ZYME 1000™◊ (Z4): 1 tablet with meals.

OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

MG/K-ZYME™◊ (Z11): 1 tablet with meals.

§Currently unavailable

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CATEGORY IX: Hepatic Detoxication

Hepatic Detoxication
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Dietary and Nutritional Support for Hepatic Biotransformation:**

ClearVite™◊ Formula (K84, 95, 105, 108, 118, or 142): As directed by the ClearVite™◊ Basic Program Guide.
Hepato-Synergy™◊ (K69): 1 packet once a day or as needed.
Trizomal™◊ Glutathione (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.
Nourish Greens™◊ (K67): 1 scoop, with up to 8 ounces of water, once a day.

CATEGORY X: Sugar Metabolism

(also check for Hypoglycemia)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Sugar Metabolism:**

Proglyco-SP™◊ (K13): 1 capsule 3 times a day, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
Butyric-FBR™◊ (K133): 2 capsules once a day.

Alternative Formula

- Fibromin™◊ (K25): 1 capsule, 2 times a day, or as needed.

Additional Support Considerations:

Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.
OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
- Super EFA Complex™◊ (K8): 2 tablespoons once a day.§

Also Consider:

AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

CATEGORY XI: Peripheral Utilization of Sugars

(also check for Insulin Resistance)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Peripheral Utilization of Sugars:**

Glysen™◊ (K1), (K135): 1-2 capsules with meals, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
Glycoberine-MX™◊ (K126): 1 capsule with a meal.

Alternative Formula

- Glycoberine™◊ Active (K127): 1 capsule with a meal.

EnteroVite™◊ (K100): 1-2 capsules with meals.

Alternative Formula

- Butyric-FBR™◊ (K133): 1-2 capsules with meals.

Additional Support Considerations:

Glysen-Synergy™◊ (K79): 1 packet, once a day, or as needed.
OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formula

- OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.

Resvero Active™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) 2 times a day.

CATEGORY XI: Peripheral Utilization of Sugars (Cont'd)

Also Consider:

AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.
Oxicell™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.

Alternative Formula

- Super Oxicell™◊ (KR23): Apply in the morning, afternoon, and evening.

CATEGORY XII: Adrenal

(also check for Adrenal Hypofunction)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Adrenal Function:**

Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

*May Also Need to Support Sugar Metabolism (see Category X):**

Proglyco-SP™◊ (K13): 1-2 capsules with meals.
OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
- Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

CATEGORY XIII: Adrenal

(also check for Adrenal Hyperfunction)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Adrenal Function:**

Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

*May Also Need to Support the Peripheral Utilization of Sugars (see Category XI):**

Glysen™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.

Alternative Formula

- OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

Additional Formula

- Neurosera™◊ (K136): 1-2 teaspoons (5-10 mL) once a day.

Also Consider:

AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

CATEGORY XIV: Electrolyte and pH Balance

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Electrolyte and pH Balance:**

Electro-pH™◊ Complex (K86): 1 scoop with up to 8 ounces of water.

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CATEGORY XV: Thyroid

(also check for Hypofunction)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Thyroid Function:**
THYROXAL™◊ (K12): 1-2 capsules with meals.

*Nutritional Support for Thyroid Immune Pathways:**
THYRO-FLM™◊ (K137): 2 capsules once a day.

*Nutritional Support for the Thyroid:**
THYRO-CNV™◊ (K9): 1-2 capsules with meals.

Thyroid Underconversion

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for T3:**
THYRO-CNV™◊ (K9): 1-2 capsules with meals.
ADAPTOCRINE™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:
ADRENACALM™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

CATEGORY XVI: Thyroid

(also check for Hyperfunction)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Thyroid:**
TRIZOMAL™◊ GLUTATHIONE (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.

Alternative Formulas
- AC-GLUTATHIONE 250™◊ (K117): 1 capsule 1-2 times a day.
- AC-GLUTATHIONE™◊ (K88): 1-2 capsules 1-2 times a day.

CLEARVITE™◊ Formula (K84, 95, 105, 108, 118, or 142): As directed by the CLEARVITE™◊ Basic Program Guide.
NITRIC BALANCE™◊ (K62), (K68), (K82): 1 teaspoon once a day.

Additional Support Considerations:
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas
- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

ULTRA-D 5000™◊ (K78): 1-4 teaspoons, once a day, with meals.

Alternative Formulas
- ULTRA-D COMPLEX™◊ (K35): 1-4 teaspoons, once a day, with meals.
- AD-PRO™◊ (K119): 1 softgel once a day.
- ADEK-PRO™◊ (K102): 1-3 softgels once a day.

For additional vitamin D, consider LIQUA-D™◊ (K87): 1 drop once a day.

§Currently unavailable

Also Consider:
OXICELL™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.

Alternative Formula
- SUPER OXICELL™◊ (KR23): Apply in the morning, afternoon, and evening.

CATEGORY XVII: Prostate (Males Only)

(also check for Hypertrophy)
FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for the Prostate:**
PROSTA-DHT™◊ (K6): 1-2 capsules with meals.

Additional Support Considerations:
FIBROMIN™◊ (K25): 1-3 capsules with meals.
ESTROVITE™◊ (K5): 1-2 capsules with meals.
BLADDER SERENE™◊ (K138): 1 scoop (2.6 g), with up to 6 ounces of water, once a day.
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas
- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

CATEGORY XVIII: Andropause (Males Only)

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Testosterone Levels:**
OPTICRINE™◊ (K3): 1-2 capsules with meals.
DHEA-25C™◊ (K141): 1 capsule once a day.

Alternative Formula
- DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

Elevated Estrogen

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Estrogen Levels:**
ESTROVITE™◊ (K5): 1-2 capsules with meals.
GLYSEN™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.

Alternative Formula
- OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

FIBROMIN™◊ (K25): 1 capsule, 2 times a day, or as needed.
RESVERO ACTIVE™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) once a day.

Alternative Formula
- RESVERO-SE™◊ ACTIVE (K125): 1 softgel once a day.

Also Consider:
TESTANEX™◊ (KR17), (KR74): Apply 2 times a day.

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CATEGORY XIX: Menstruating Women

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Progesterone Function:**

PROGESTAID™◊ (K4): 1-2 capsules with meals.
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

ESTROVITE™◊ (K5): 1-2 capsules with meals.

Adrenal Hypofunction *(see Category XI)*

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Adrenal Function:**

PROGLYCO-SP™◊ (K13): 1-2 capsules with meals.
ADAPTOCRINE™◊ (K2), (K124): 1-3 capsules with meals.
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

Also Consider:

ADRENASTIM™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

CATEGORY XX: Menopausal Women

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Progesterone Function:**

PROGESTAID™◊ (K4): 1-2 capsules with meals.
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

Estrogen Metabolism

FOR TREATMENT OF SYMPTOMS, OR IF YOU SUSPECT ANY DISEASE OR CONDITION: Seek appropriate conventional medical management and drug administration by a physician.

*Nutritional Support for Estrogen Metabolism:**

ESTROVITE™◊ (K5): 1-2 capsules with meals.
OMEGACO3-SE™◊ (K113): 2 softgels 1-2 times a day.

Alternative Formulas

- OMEGACO3™◊ (K7): 1 tablespoon 1-2 times a day.
- SUPER EFA COMPLEX™◊ (K8): 1-2 tablespoons with meals.§

DHEA-25C™◊ (K141): 1 capsule once a day.

Alternative Formula

- DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

§Currently unavailable

*Nutritional Support for Progesterone Function:**

PREGNENOLONE (K19): 10 drops (0.255 mL) 1-3 times a day.

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